

Navigating CMS Policies: Compliance, Billing, and Reimbursement Best Practices

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Navigating CMS Policies: Compliance, Billing, and Reimbursement Best Practices

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Objectives

01

Understand the latest **CMS policy updates** for Medicare and Medicaid.

02

Implement **best practices for billing, coding, and documentation compliance**.

03

Identify **risk areas and audit preparedness strategies**.

04

Navigate **reimbursement models and optimize revenue cycle efficiency**.

05

Utilize **practical compliance tools and resources**.



HELLO!

Jennifer McNamara,
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As CEO of Healthcare Inspired LLC and a Compliance Consultant, Jennifer is a nationally recognized expert in healthcare management and compliance. With a dynamic presence as a speaker, podcaster, and auditor, Jennifer inspires positive change in the healthcare industry.

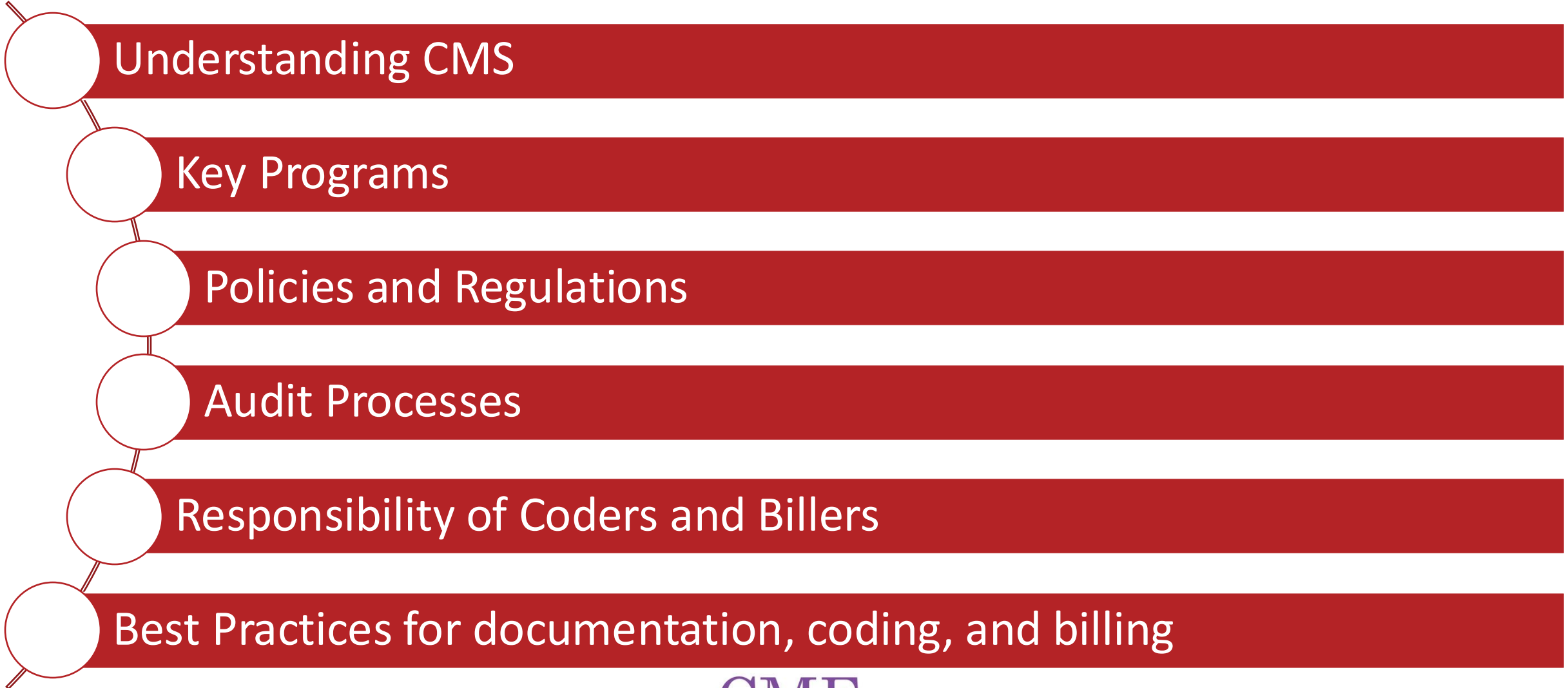


Welcome! Today, we'll navigate the complexities of CMS policies, focusing on compliance, accurate billing and coding, risk mitigation, and reimbursement strategies. Let's dive into best practices that enhance documentation, optimize revenue cycle efficiency, and ensure audit readiness!

Disclaimer

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Objectives



Who is CMS?

The Centers for Medicare & Medicaid Services (CMS) is a U.S. federal agency within the Department of Health and Human Services. CMS administers major healthcare programs like Medicare, Medicaid, the Children's Health Insurance Program (CHIP), and the Health Insurance Marketplace. It works to ensure that millions of Americans have access to affordable, high-quality healthcare. CMS also sets standards for healthcare providers, enforces regulations, and strives to improve healthcare outcomes and reduce costs.

- **Policy and Regulation:** Develops and implements policies and regulations to ensure program efficiency and integrity.
- **Funding and Reimbursement:** Manages federal funding and reimbursement for Medicare and Medicaid services.
- **Quality Improvement:** Initiates programs to improve healthcare quality and patient safety.
- **Data and Research:** Collects and analyzes healthcare data to inform policy and improve public health.
- **Compliance and Audits:** Conducts audits and compliance checks to prevent fraud and abuse in healthcare programs.

CMS: Who They Are and Why It Matters

Centers for Medicare & Medicaid Services (CMS): Federal agency overseeing Medicare, Medicaid, and CHIP. Key Responsibilities include:

Regulatory updates and compliance enforcement

Payment model changes

Fraud prevention and audit oversight

Key CMS Updates for 2024-2025



Medicare Fee Schedule
Adjustments

Telehealth and Remote Patient
Monitoring Changes

ICD-10 and CPT Code Updates

Evaluation & Management (E/M)
Documentation Guidelines

Medicaid Expansion and State-
Specific Variations

History of CMS

Establishment:

- Signed into law on July 30, 1965, by President Lyndon B. Johnson.
- Original Medicare included Part A (Hospital Insurance) and Part B (Medical Insurance), now called “Original Medicare.”

Expansion:

- 1972: Medicare extended to cover the disabled, people with ESRD, and people 65 or older opting for Medicare.
- Introduction of more benefits, including prescription drug coverage.

Medicaid Evolution:

- Initially for those receiving cash assistance.
- Now covers low-income families, pregnant women, people of all ages with disabilities, and those needing long-term care.
- States can customize Medicaid services.

History of CMS

Medicare Part D:

- Medicare Prescription Drug Improvement and Modernization Act of 2003 (MMA).
- Introduction of Medicare Advantage Plans (Part C).
- Optional prescription drug benefit (Part D) initiated in 2006.

Children's Health Insurance Program (CHIP):

- Established in 1997.
- Provides health insurance to nearly 11 million uninsured American children.
- Covers children from uninsured working families in all states, DC, and territories.

Affordable Care Act (ACA):

- Introduced in 2010.
- Established the Health Insurance Marketplace.
- Enhanced coordination between Medicare and Medicaid for better service quality..

Medicare Basics

Part A: Hospital Insurance

- **Coverage:** Inpatient hospital stays, skilled nursing facility care, hospice care, and some home health care.
- **Cost:** Usually free if you or your spouse paid Medicare taxes while working.

Part B: Medical Insurance

- **Coverage:** Doctor visits, outpatient care, preventive services, and some home health care.
- **Cost:** Monthly premium, with deductible and coinsurance.

Part C: Medicare Advantage

- **Coverage:** Combines Part A and Part B, often includes Part D, and may offer additional benefits like vision, dental, and hearing.
- **Offered By:** Private insurance companies approved by Medicare.

Part D: Prescription Drug Coverage

- **Coverage:** Helps cover the cost of prescription drugs.
- **Cost:** Varies by plan, includes premiums, deductibles, and copayments.

Medicare Part C

Medicare Advantage (MA) Program

- Established by the Balanced Budget Act of 1997 and renamed under the Medicare Prescription Drug, Improvement, and Modernization Act of 2003.

Plan Types

- Coordinated Care Plans Health Maintenance Organizations (HMOs), Provider Sponsored Organizations (PSOs), and Preferred Provider Organizations (PPOs).
- Medical Savings Account (MSA) Plans**: Combine a high-deductible health plan with a savings account.
- Private Fee-for-Service (PFFS) Plans Allow beneficiaries to see any provider that accepts Medicare.

Eligibility and Enrollment

- Eligibility Generally for those who are eligible for Medicare Part A and enrolled in Part B.
- Enrollment Periods: Annual enrollment periods, special enrollment periods for qualifying events.

Updates and Changes

- CMS periodically revises regulations to clarify participation requirements and strengthen beneficiary protections.

Quality and Performance

- CMS continually works to improve plan quality and remove poor performers from the program.

Medicaid History

Medicaid:

- **Authorized by Title XIX of the Social Security Act:** Medicaid was signed into law in 1965 alongside Medicare.
- **Purpose:** Provides health coverage for low-income individuals, families, children, pregnant women, seniors, and people with disabilities.
- **State Variations:** Each state administers its Medicaid program differently within federal guidelines, leading to variations in coverage.

Children's Health Insurance Program (CHIP):

- **Established in 1997:** Provides federal matching funds to states to cover children in families with incomes too high for Medicaid but who can't afford private insurance.
- **Coverage Expansion:** Nearly every state covers children up to at least 200% of the Federal Poverty Level (FPL).

Basic Health Program (BHP):

- **Enacted by the Affordable Care Act:** Provides states the option to offer health benefits to low-income residents who might otherwise purchase coverage through the Health Insurance Marketplace.

Affordable Care Act (ACA):

- **Medicaid Expansion:** Starting in 2014, the ACA allowed states to expand Medicaid eligibility to individuals under 65 with incomes below 133% of the FPL.
- **Provisions:** Standardizes eligibility and benefits, enhances information technology systems, coordinates with insurance exchanges, and includes community-based services, quality improvements, and program integrity measures.

CMS Policy vs Regulation

CMS Rulemaking:

Purpose: CMS regulations establish or modify the way CMS administers its programs, impacting providers, suppliers, and beneficiaries.

Publication: Regulations are published in the daily national "Federal Register," which is available online and at many public libraries.

Proposed Regulations:

Proposed Rule: Announces CMS's intent to issue or modify regulations and solicits public comments during a designated comment period.

Public Participation: Anyone can comment on proposed regulations, which CMS considers before finalizing the rule.

Final Regulations:

Development: After reviewing public comments, CMS may publish a final regulation.

Effective Date: Significant regulations are typically effective 60 days after publication, while others may be effective 30 days after publication.

CMS Policy vs Regulation

CMS Regulations: Official rules issued by the Centers for Medicare & Medicaid Services to guide the administration of its programs. These regulations establish or modify how CMS manages its services and impacts healthcare providers, suppliers, and beneficiaries.

Importance of Regulations

Consistency and Standardization: Ensures uniform standards and practices across the healthcare system.

Quality and Safety: Protects patients by ensuring healthcare providers meet specific standards.

Compliance and Enforcement: Helps prevent fraud and abuse within CMS programs.

Public Trust: Promotes transparency and accountability in healthcare delivery.

Framework



How CMS Regulations are Structured:

- **Title and CMS Number:** Each regulation has a specific title and CMS number for reference.
- **Preamble:** Provides background, purpose, and summary of the regulation.
- **Regulatory Text:** Detailed provisions and requirements that healthcare providers must follow.
- **Effective Dates:** Specifies when the regulation goes into effect.

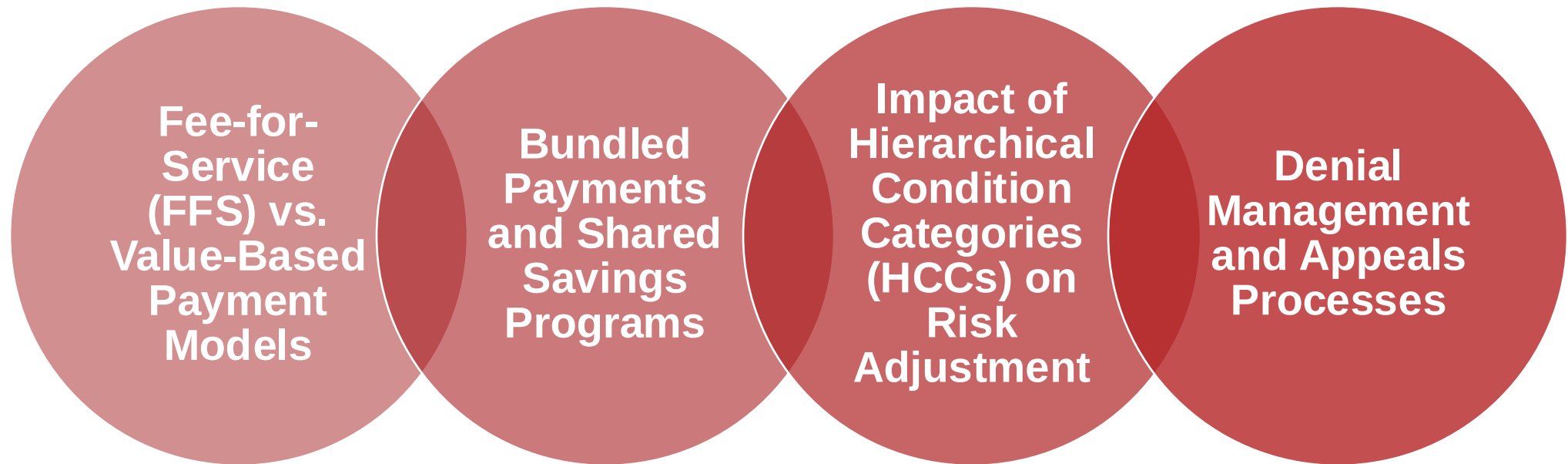


Ensuring Compliance:

- **Monitoring and Audits:** CMS conducts regular audits and compliance checks to ensure adherence to regulations.
- **Education and Training:** Provides guidance and training to help healthcare providers understand and implement regulations.
- **Enforcement Actions:** Includes penalties, corrective action plans, and sanctions for non-compliance.



Maximizing Revenue with the Right Payment Model



Ensuring Accuracy in Billing and Coding

Documentation Accuracy: Importance of complete, consistent, and timely documentation.



Medical Necessity: Align coding with clinical justifications.

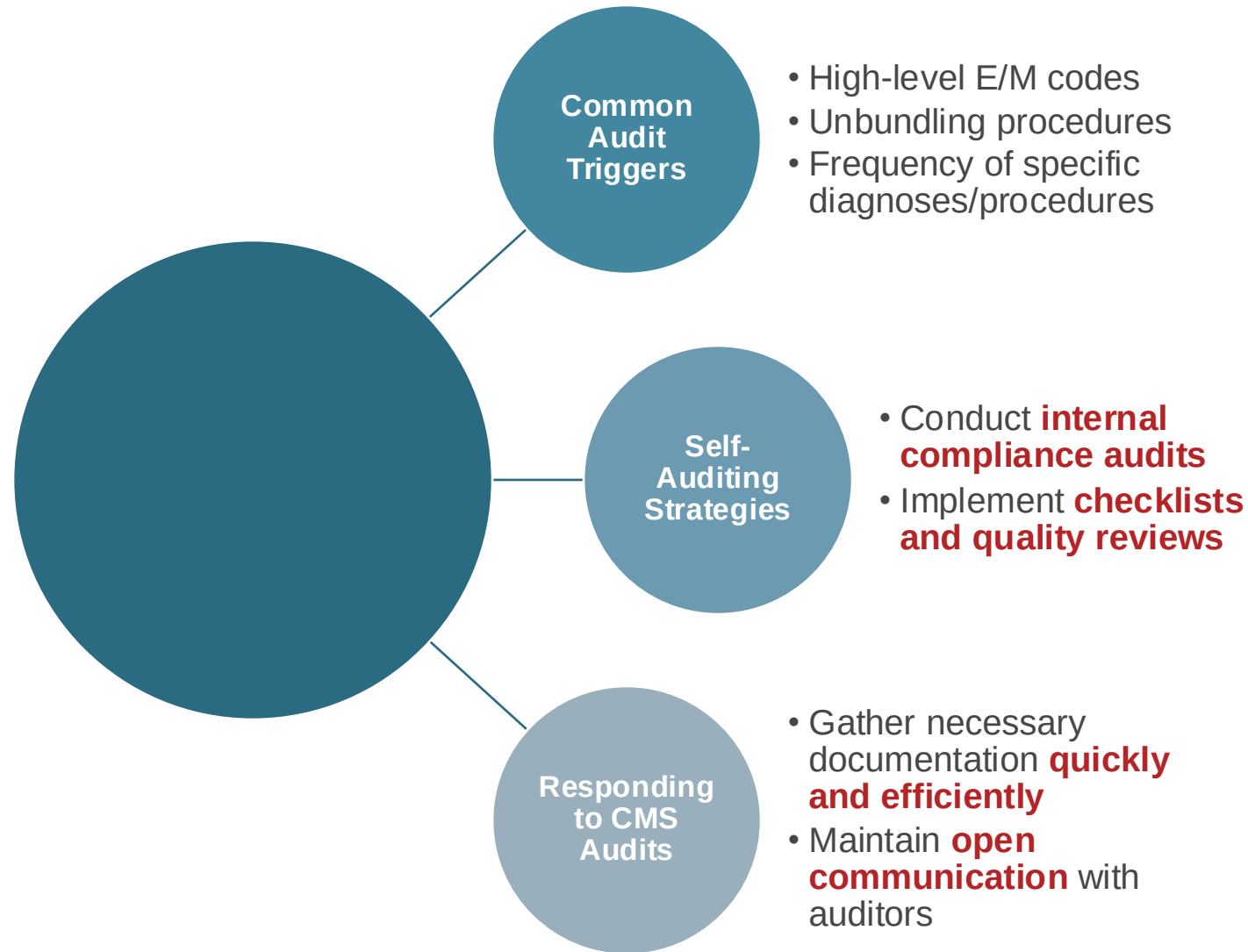


Modifier Usage: Avoid common modifier errors (e.g., -25, -59).

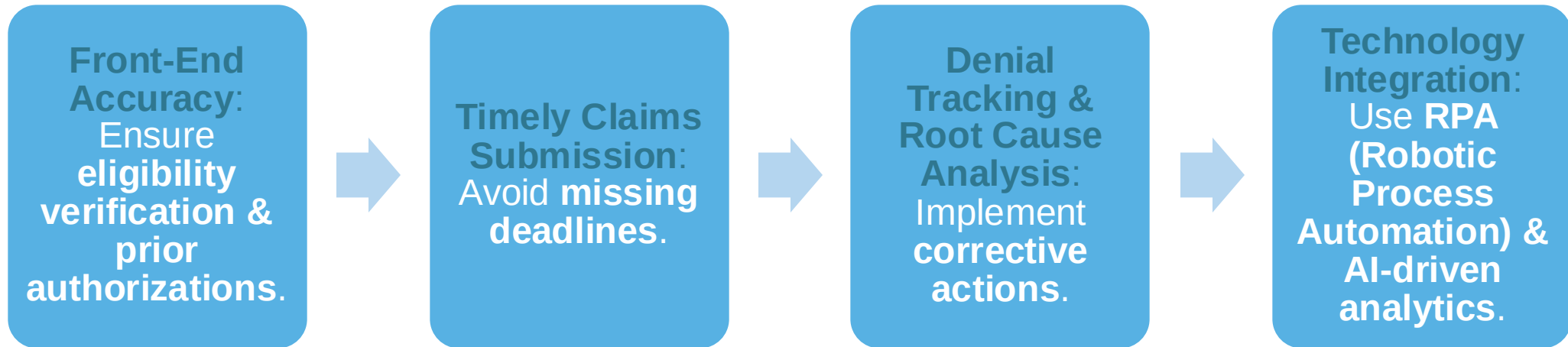


Upcoding & Downcoding Risks: Stay within CMS guidelines.

Reducing Risks and Preparing for Audits



Boosting Financial Health Through Process Improvement



CMS Audits

Purpose

- **Ensuring Compliance:** Audits ensure that healthcare providers comply with Medicare and Medicaid regulations and guidelines. This includes proper billing, coding, and documentation practices.
- **Preventing Fraud and Abuse:** They help identify and prevent fraudulent activities and abuse within the healthcare system. This includes detecting improper billing practices and unauthorized use of funds.
- **Quality of Care:** Audits assess the quality of care provided to beneficiaries. This ensures that patients receive appropriate, necessary, and high-quality healthcare services.
- **Financial Integrity:** They ensure the financial integrity of Medicare and Medicaid programs by verifying that payments made to providers are accurate and justified.
- **Program Improvement:** Insights gained from audits can lead to improvements in policies, procedures, and overall program administration, enhancing the effectiveness and efficiency of healthcare delivery.

Different Types of CMS Audits

Comprehensive Error Rate Testing (CERT) Audit:

- **Purpose:** To measure the accuracy of Medicare Fee-for-Service (FFS) payments.
- **Focus:** It involves a random sampling of claims to determine if they were paid correctly according to Medicare coverage, coding, and billing rules.

Recovery Audit Contractor (RAC) Audit:

- **Purpose:** To identify and correct improper payments made on claims of healthcare services provided to Medicare beneficiaries.
- **Focus:** RAC auditors review claims to find overpayments and underpayments.

Medicare Advantage (MA) Audits:

- **Purpose:** To ensure the accuracy of risk adjustment data submitted by Medicare Advantage plans.
- **Focus:** These audits check if health plans receive appropriate payments based on the health status of their enrollees.

Zone Program Integrity Contractor (ZPIC) Audits:

- **Purpose:** To identify and investigate potential fraud, waste, and abuse in Medicare.
- **Focus:** ZPIC audits target high-risk areas and providers with suspicious billing patterns or a high volume of claims.

Unified Program Integrity Contractor (UPIC) Audits:

- **Purpose:** To identify and prevent fraud, waste, and abuse in both Medicare and Medicaid.
- **Focus:** UPICs conduct investigations and audits to ensure program integrity across both federal healthcare programs.

Targeted Probe and Educate (TPE) Audits:

- **Purpose:** To reduce provider burden and improve claim accuracy.
- **Focus:** TPE audits involve a focused review of claims followed by one-on-one education to help providers improve their billing practices and prevent future errors..

Comprehensive Error Rate Testing (CERT)

Claim Type	Improper Payment Rate	Improper Payment Amount (2)
Overall	7.38%	\$31.23 B
Part A Providers (excluding Hospital Inpatient Prospective Payment System (IPPS))	7.75%	\$14.22 B
Part B Providers	10.03%	\$10.99 B
Hospital IPPS	3.36%	\$4.08 B
Durable Medical Equipment, Prosthetics, Orthotics, and Supplies	22.51%	\$1.95 B

<https://www.cms.gov/data-research/monitoring-programs/improper-payment-measurement-programs/comprehensive-error-rate-testing-cert/cert-reports>

Responding

Request for Documentation

- Initial Request: Empower AI, Inc. selects a random sample of claims monthly and sends a request letter for supporting documentation.
- Required Documents: Orders, refill requests, medical records, proof of delivery, etc.
- Response Timeline: Submit documentation within 45 days; follow-up letters/calls at Days 21, 30, 35, 45, 49, 60, and score claim as an error at Day 76 if not received.

Subsequent Requests

- Additional Info Needed: Shorter response time.
- Follow-Up: Day 1 and Day 10 letters and calls; claim back in review on Day 16.

Verify Contact Info

- Ensure Accuracy: Verify supplier name, NPE/NPI, address on the CERT Provider website.

Responsibility to Respond

- Legal Requirement: Respond even if records are unavailable; failure results in overpayment recoupment.

Submitting Documentation

- Preferred Method: Fax with bar-coded cover sheet; ensure legibility and completeness. Contact CERT Office if unable to fax.

Recovery Audit Contractor (RAC) Audit



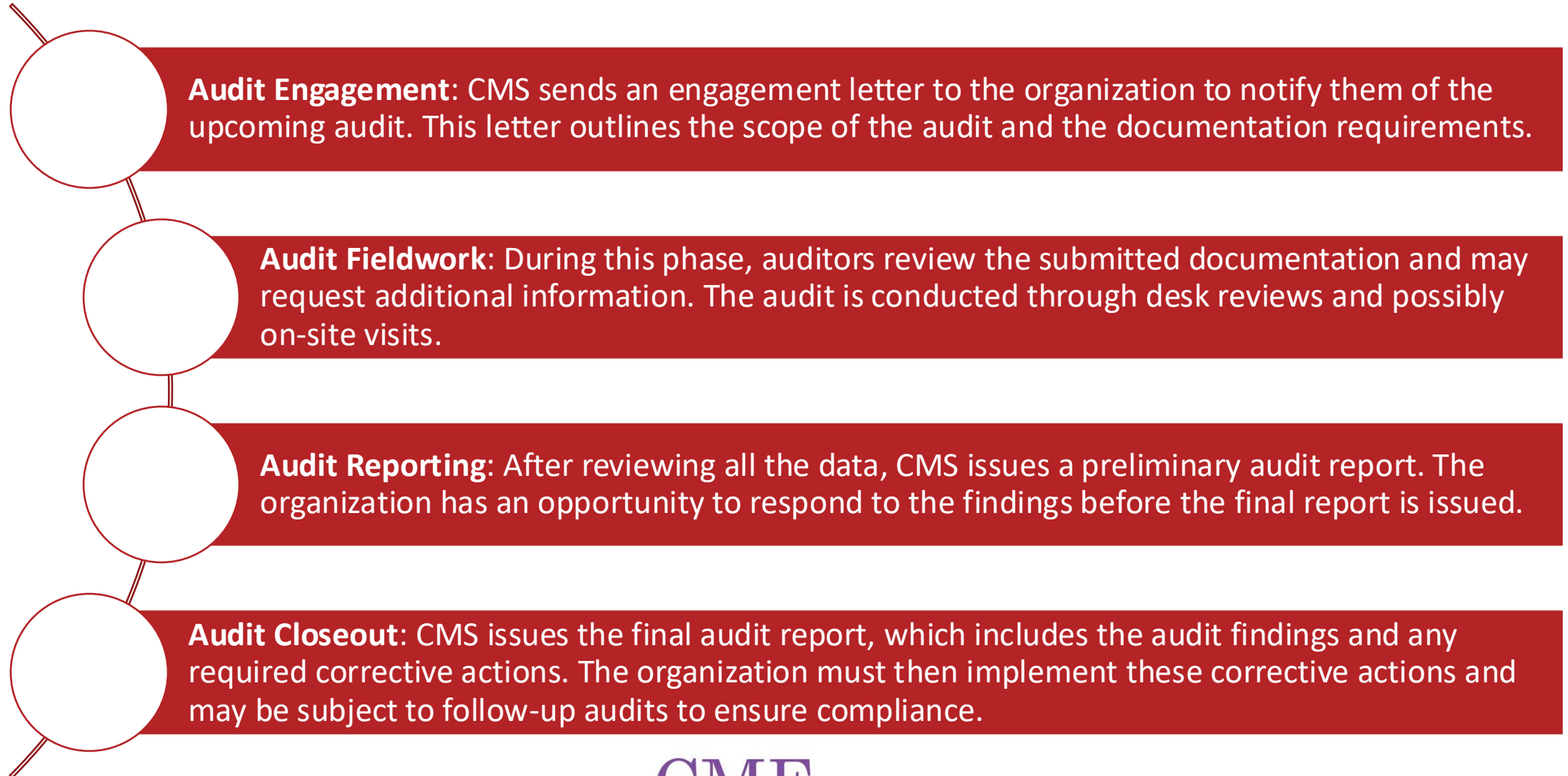
<https://www.cms.gov/data-research/monitoring-programs/medicare-fee-service-compliance-programs/medicare-fee-service-recovery-audit-program>

Proposed

<u>Issue Name</u> ▼	<u>Review Type</u>	<u>Provider Type</u>	<u>MAC Jurisdiction</u>	<u>Date Proposed</u>
<u>1A319-Non-Physician Billed Without Correct Assistant at Surgery Modifier: Incorrect Coding</u>	Automated	Professional Services	All A/B MACs	2024-05-07
<u>2A318-Assistant at Surgery Services Billed Without Correct Payment Modifiers: Incorrect Coding</u>	Complex	Professional Services	All A/B MACs	2024-05-07

- Post Payment Reviews:** RAC reviews claims after payment.
- Issue Identification:** Specific issues are pursued only if approved through CMS's "new issue review" process.
- Non-Posted Issues:** Small samples may be requested for potential new issues.
- Stay Informed:** Suppliers should regularly check the Recovery Auditor's website for new and pending issues. The CMS Provider Resources webpage lists all pending approval issues.

Medicare Advantage (MA) Audits



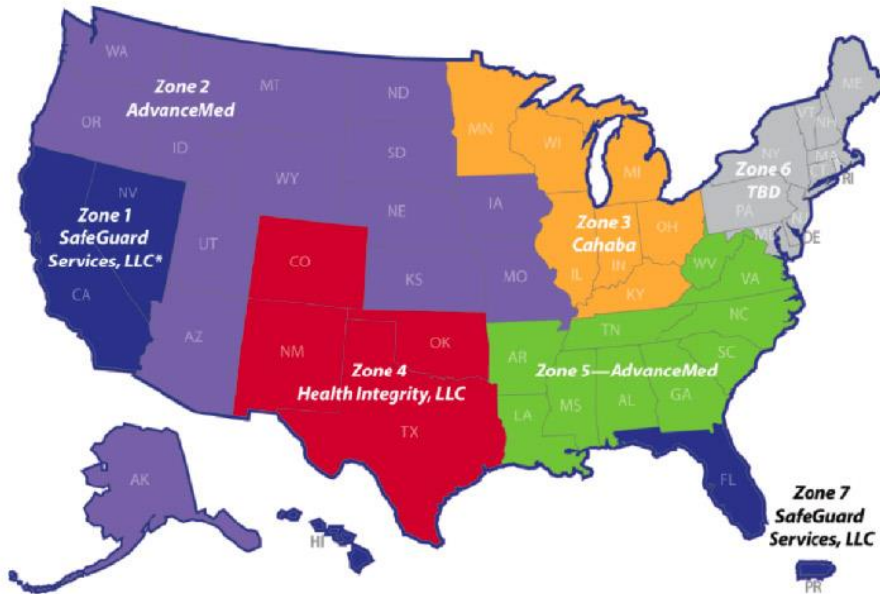
Zone Program Integrity Contractor (ZPIC) Audits

The goal of the UPIC program is to identify and investigate suspected cases of fraud, waste, and abuse in Medicare and Medicaid. The program ensures that inappropriate payments are recouped and that the integrity of the Medicare and Medicaid programs is maintained.

- **Examples of Fraud:**
 - **Billing for services not furnished:** Submitting claims for services that were never provided to patients.
 - **Deliberate duplicate billing:** Submitting multiple claims for the same service to receive multiple payments.
 - **Altering claims or medical records:** Modifying documents to obtain higher reimbursement.
 - **Kickbacks or rebates:** Soliciting, offering, or receiving payments for patient referrals.
 - **Billing non-covered services as covered:** Submitting claims for services that are not eligible for coverage as if they were.

Zone Program Integrity Contractor (ZPIC) Audits

Chart A: Breakdown of ZPIC Geographic Region



Unusual Billing Practices

Inaccurate Billing Practices

Incomplete or Inconsistent Records

Complaints or Referrals

Disorganized Documentation

Unified Program Integrity Contractor (UPIC) Audits

UPIC Region	Contractor	Covered States/Territories
Midwest	<u>CoventBridge Group</u>	IA, KS, MO, and NE
Southwestern		CO, NM, OK, TX, AR, LA, and MS
Western	<u>Qlarant</u>	Am. Samoa, Guam, N. Mariana Is., AK, AZ, CA, HI, ID, MT, NV, ND, OR, SD, UT, WA, and WY
South-Eastern	SafeGuard Services LLC (SGS)	WV, VA, NC, SC, TN, AL, GA, and FL
Northeastern	SafeGuard Services LLC (SGS)	ME, VT, NH, MA, RI, CT, NY, PA, NJ, DE, MD, and District of Columbia

Role and Purpose

- **Detection, Deterrence, and Prevention:** UPICs focus on identifying and mitigating fraud, waste, and abuse in Medicare and Medicaid programs.
- **Scope of Activities:** Coverage includes Medicare Parts A, B, DME, Home Health and Hospice (HH+H), Medicaid, and the Medi-Medi program.

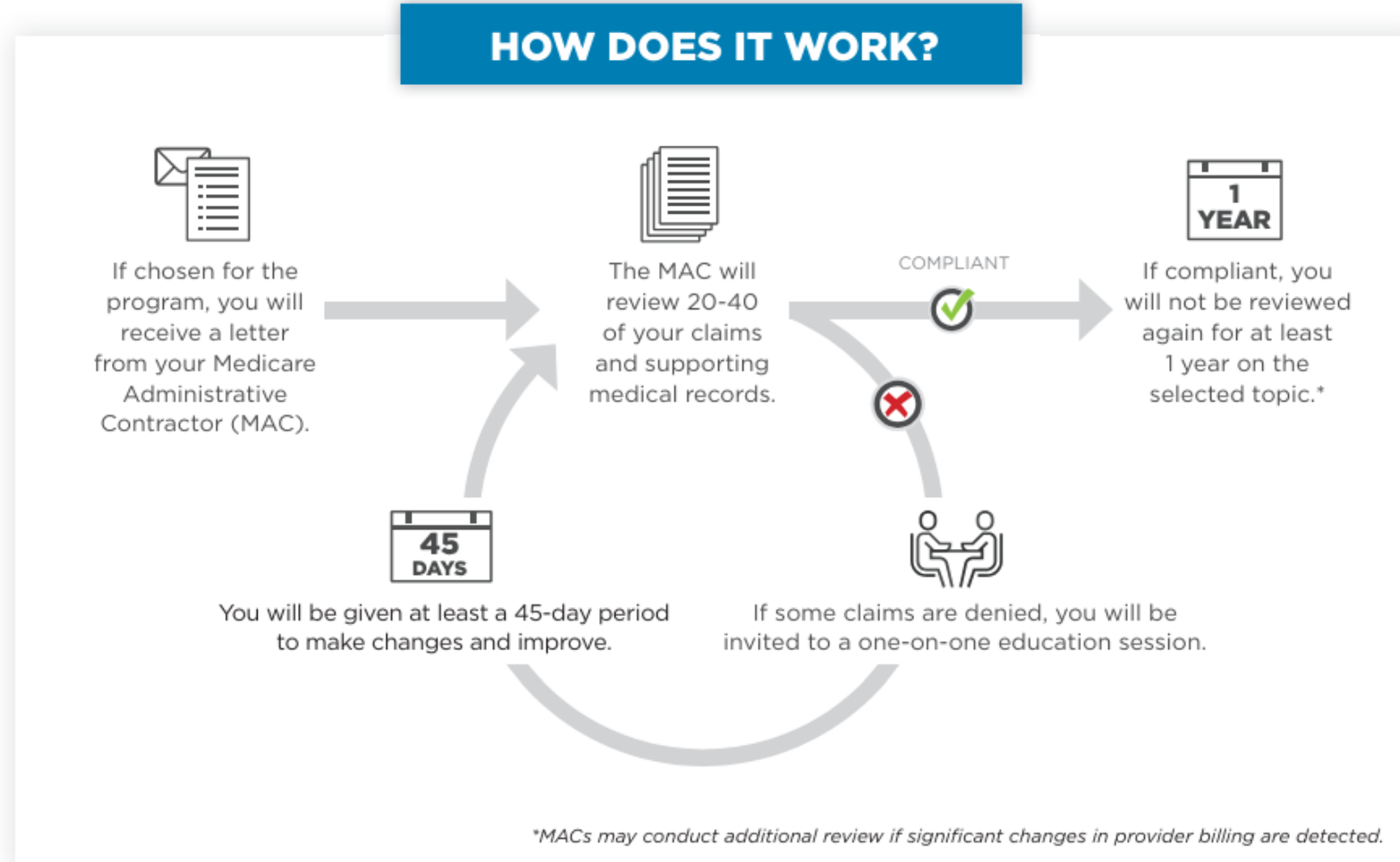
Geographical Jurisdictions

- **Nationwide Coverage:** UPICs operate in five distinct geographical regions across the United States.
- **Integrated Functions:** Combines roles previously managed by ZPIC, PSC, and MIC contracts for enhanced efficiency.

Impact and Importance

- **Financial Integrity:** Ensures proper use of healthcare funds and compliance with federal regulations.
- **Patient Care:** Protects resources to guarantee that patients receive necessary care without financial fraud or abuse.

Targeted Probe and Educate (TPE) Audits



<https://www.cms.gov/data-research/monitoring-programs/medicare-fee-service-compliance-programs/medical-review-and-education/targeted-probe-and-educate-tpe>

Compliance Tips

Allergy Services	Ambulance Services	Ambulatory Surgical Centers	Annual Wellness Visits	Anticancer Drugs	Bacterial Cultures	Blood Counts	Canes & Crutches
Cataract Services	Chiropractic Services	Commodes	CORF Services	CPAP Devices	Diabetic Shoes	Diabetic Supplies	Echography & Sonography
Enteral Nutrition	Enteral Nutrition Pumps	ESRD Clinic Services	Evaluation & Management	Hip & Knee Replacements	Home Health Services	Hospice Services	Hospital Beds
Immunosuppressive Drugs	Infusion Pumps	Inpatient Rehabilitation Services	Lenses	Lipid Panels	Lower Limb Orthoses	Lower Limb Prostheses	Manual Wheelchairs
Nebulizers	Negative Pressure Wound Therapy	Ostomy Supplies	Other Lab Tests	Oxygen	Parenteral Nutrition	Patient Lifts	Physical Therapy
Pneumatic Compression Devices	Podiatry	Pressure Reducing Support Surfaces	Psychiatric Care	Respiratory Assist Devices	Sleep Studies	SNF Services	Spinal Orthoses
Surgical Dressings	TENS Units	Tracheostomy Supplies	Urinalysis	Urological Supplies	Venipuncture	Ventilators	Walkers



<https://www.cms.gov/Outreach-and-Education/Medicare-Learning-Network-MLN/MLNProducts/medicare-provider-compliance-tips/medicare-provider-compliance-tips.html>

HCPCS & CPT Codes

[Article: Billing and Coding: Routine Foot Care](#) has the most current HCPCS and CPT codes. This page includes local coverage information that applies to some providers. Select your MAC's Article from the search results and review the article or policy to see if the coverage information applies to you.

Background

According to the [2022 Medicare Fee for Service \(FFS\) Supplemental Improper Payment Data](#), the improper payment rate for podiatry care is 14.9%, with a projected improper payment amount of \$238.3 million.

Denial Reasons

For the 2021 reporting period, insufficient documentation accounted for 80.9% of improper payments for podiatric providers, while incorrect coding errors (18.4%), medical necessity (0.5%), and ["other" errors](#) (0.2%) caused other improper payments.

Preventing Denials

We don't cover foot care services we consider routine, including:

- Cutting or removing corns and calluses
- Trimming, cutting, clipping, or debriding nails
- Other hygienic and preventive maintenance care (for example, cleaning and soaking the feet, using skin creams to support skin tone of either ambulatory or bedfast patients, and other services done in the absence of localized illness, injury, or symptoms involving the foot)

Exceptions to Routine Foot Care Exclusion

Necessary and Integral Part of Otherwise Covered Services

In certain circumstances, we may cover services ordinarily considered routine if they're necessary and integral to otherwise covered services (for example, diagnosis and treatment of ulcers, wounds, or infections).

Internet Only Manual IOM



<u>100-01</u>	Medicare General Information, Eligibility and Entitlement Manual
<u>100-02</u>	Medicare Benefit Policy Manual
<u>100-03</u>	Medicare National Coverage Determinations (NCD) Manual
<u>100-04</u>	Medicare Claims Processing Manual <u>Chapter 12 - Physicians/Nonphysician Practitioners</u>
<u>100-05</u>	Medicare Secondary Payer Manual
<u>100-06</u>	Medicare Financial Management Manual
<u>100-07</u>	State Operations Manual
<u>100-08</u>	Medicare Program Integrity Manual
<u>100-09</u>	Medicare Contractor Beneficiary and Provider Communications Manual

Internet Only Manual IOM

30 - Correct Coding Policy

- 30.1 - Digestive System (Codes 40000 - 49999)
- 30.2 - Urinary and Male Genital Systems (Codes 50010 - 55899)
- 30.3 - Audiology Services
- 30.4 - Cardiovascular System (Codes 92950-93799)
- 30.5 - Payment for Codes for Chemotherapy Administration and Nonchemotherapy Injections and Infusions
- 30.6 - Evaluation and Management Service Codes - General (Codes 99202 - 99499)
 - 30.6.1 - Selection of Level of Evaluation and Management Service

190 - Medicare Payment for Telehealth Services

- 190.1 - Background
- 190.2 - Eligibility Criteria
- 190.3 - List of Medicare Telehealth Services
 - 190.3.1 - Telehealth Consultation Services, Emergency Department or Initial Inpatient versus Inpatient Evaluation and Management (E/M) Visits
 - 190.3.2 - Telehealth Consultation Services, Emergency Department or Initial Inpatient Defined
 - 190.3.3 - Follow-Up Inpatient Telehealth Consultations Defined

Surgeons and Global Surgery

- 40.1 - Definition of a Global Surgical Package
- 40.2 - Billing Requirements for Global Surgeries
- 40.3 - Claims Review for Global Surgeries
- 40.4 - Adjudication of Claims for Global Surgeries
- 40.5 - Postpayment Issues
- 40.6 - Claims for Multiple Surgeries
- 40.7 - Claims for Bilateral Surgeries
- 40.8 - Claims for Co-Surgeons and Team Surgeons
- 40.9 - Procedures Billed With Two or More Surgical Modifiers

NCCI



How to Use the Medicare National Correct Coding Initiative (NCCI) Tools



National Correct Coding Initiative (NCCI) edits

- [Medicare Correspondence Language Policy Manual](#)
- [Medicare NCCI Add-on Code Edits](#)
- [Medicare NCCI FAQ Library](#)
- [Medicare NCCI Medically Unlikely Edit \(MUE\) Archive](#)
- [Medicare NCCI Medically Unlikely Edits \(MUEs\)](#)
- [Medicare NCCI Policy Manual](#)
- [Medicare NCCI Procedure to Procedure \(PTP\) Edits](#)

Medicare NCCI Procedure to Procedure (PTP) Edits

National Correct Coding Initiative (NCCI) Procedure-to-Procedure (PTP) edits prevent inappropriate payment of services that should not be reported together. Each edit has a Column One and Column Two HCPCS/CPT code. If a provider reports the two codes of an edit pair for the same beneficiary on the same date of service, the Column One code is eligible for payment, but the Column Two code is denied unless a clinically appropriate NCCI PTP-associated modifier is also reported.

Quarterly Version Update Changes

CMS posts changes to each of its NCCI PTP published edit files on a quarterly basis. This includes additions, deletions, and modifier indicator quarterly changes to PTP column one/column two correct coding edits and the PTP mutually exclusive code edits for Practitioners and Hospital Outpatient PPS in the Outpatient Code Editor.

Related Downloads

2024 Quarter 1 Edit Files:

Hospital PTP Edits

[Hospital PTP Edits v300r0 \(445,176 Records\) 0001A/0591T - 24320/G0471 \(ZIP\)](#) - Effective Jan. 01, 2024; Posted Dec. 1, 2023

[Hospital PTP Edits v300r0 \(444,936 Records\) 24330/0213T - 36221/G0471 \(ZIP\)](#) - Effective Jan. 1, 2024; Posted Dec. 1, 2023

[Hospital PTP Edits v300r0 \(445,037 Records\) 36222/0596T - 61343/G0471 \(ZIP\)](#) - Effective Jan. 1, 2024; Posted Dec. 1, 2023

[Hospital PTP Edits v300r0 \(437,776 Records\) 61345/0213T - U0003/U0004 \(ZIP\)](#) - Effective Jan. 1, 2024; Posted Dec. 1, 2023

Related Links

[The Frequently Asked Questions and Answers \(FAQs\) and the NCCI Policy Manual for Medicare Services](#) provide information about NCCI edits.

NCCI

A	B	C	D	E	F	G	H
A	B	C	D	E	F	G	H
CPT only copyright 2023 American Medical Association. All rights reserved. ▼							
Column1/Column2 Edits							
Column 1	Column 2	*=in existence prior to 1996	Effective Date	Deletion Date	Modifier	PTP Edit Rationale	
				*=no data	0=not allowed 1=allowed 9=not applicable		
99215	0074T		20050101	20050101	9	CPT Manual or CMS manual coding instruction	
99215	0115T		20070401	20071231	1	Misuse of Column Two code with Column One code	
99215	0116T		20070401	20071231	1	Misuse of Column Two code with Column One code	
99215	0359T		20141001	20181231	1	Misuse of Column Two code with Column One code	
99215	0360T		20141001	20181231	1	Misuse of Column Two code with Column One code	
99215	0361T		20141001	20181231	1	Misuse of Column Two code with Column One code	
99215	0362T		20201001	*	1	Misuse of Column Two code with Column One code	
99215	0362T		20141001	20191231	1	Misuse of Column Two code with Column One code	
99215	0363T		20141001	20181231	1	Misuse of Column Two code with Column One code	
99215	0364T		20141001	20181231	1	Misuse of Column Two code with Column One code	
99215	0365T		20141001	20181231	1	Misuse of Column Two code with Column One code	
99215	0366T		20141001	20181231	1	Misuse of Column Two code with Column One code	
99215	0367T		20141001	20181231	1	Misuse of Column Two code with Column One code	
99215	0368T		20141001	20181231	1	Misuse of Column Two code with Column One code	
99215	0369T		20141001	20181231	1	Misuse of Column Two code with Column One code	
99215	0370T		20141001	20181231	1	Misuse of Column Two code with Column One code	
99215	0371T		20141001	20181231	1	Misuse of Column Two code with Column One code	
99215	0372T		20141001	20181231	1	Misuse of Column Two code with Column One code	
99215	0373T		20201001	*	1	Misuse of Column Two code with Column One code	
99215	0373T		20141001	20191231	1	Misuse of Column Two code with Column One code	
99215	0374T		20141001	20181231	1	Misuse of Column Two code with Column One code	
99215	0469T		20201001	*	0	Misuse of Column Two code with Column One code	
99215	0469T		20171001	20191231	0	Misuse of Column Two code with Column One code	
99215	0543T		20201001	*	1	CPT Manual or CMS manual coding instruction	
99215	0543T		20200101	20200101	9	CPT Manual or CMS manual coding instruction	
99215	0544T		20201001	*	1	CPT Manual or CMS manual coding instruction	
99215	0544T		20200101	20200101	9	CPT Manual or CMS manual coding instruction	
99215	0567T		20201001	*	1	CPT Manual or CMS manual coding instruction	
99215	0567T		20200101	20200101	9	CPT Manual or CMS manual coding instruction	
99215	0568T		20201001	*	1	CPT Manual or CMS manual coding instruction	

NCCI and Modifiers

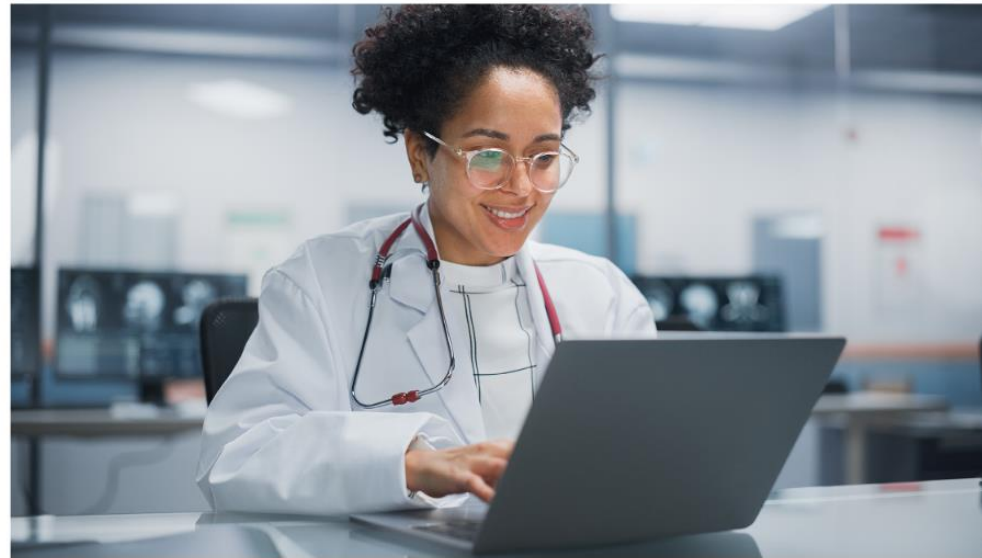
CCMIs	Definition
0 (Not Allowed)	No modifiers associated with NCCI allow you to use this PTP code pair. When no modifiers are allowed, only the Column 1 code will be paid for the same patient on the same day.
1 (Allowed)	You can use NCCI-associated modifiers with this PTP code pair when appropriate.

CCMIs	Definition
9 (Not Applicable)	This indicator is used for all code pairs that have a deletion date that is the same as the effective date. There's no active edit for this PTP code pair.

Medicare Physician Fee Schedule



How to Use the PFS Look-Up Tool



https://www.cms.gov/Outreach-and-Education/Medicare-Learning-Network-MLN/MLNProducts/Downloads/How_to_MPFS_Booklet_ICN901344.pdf

Medicare Physician Fee Schedule

HCPSC Code ▲	Modifier ▲	Short Description ◆	Proc Stat ▲	Mac Locality ▲	Non-Facility Price ◆	Facility Price ◆	Non-Facility Limiting Charge ◆	Facility Limiting Charge ◆	Conv Fact ◆
99214		Office o/p est mod 30-39 min	A	1120201	\$121.88	\$93.88	\$133.16	\$102.57	33.8872
99215		Office o/p est hi 40-54 min	A	1120201	\$171.09	\$137.85	\$186.91	\$150.61	33.8872

Status Indicators

A = Active code. Medicare pays these codes separately under the physician fee schedule (PFS), if covered. Codes with this status include RVUs and payment amounts. The presence of an A indicator doesn't mean that Medicare has made a national coverage determination about the service. A/B MACs (B) stay responsible for coverage decisions in the absence of a national Medicare policy.

B = The PFS always bundles payment for covered services into payment for other services not specified. No RVUs or payment amounts exist for these codes and Medicare never makes separate payment. When Medicare covers these services, we include payment for them in the payment for the services to which they're incident. An example is a telephone call from a hospital nurse about the care of a patient.

Payment Calculation

RVU	GPCI	CF(Conversion Factor) \$32.74
Work, Practice Expense (PE), Malpractice (MP)	Add each RVU Type X GPCI Locality	RVU/GPCI X \$32.74



Figure 1: Arithmetic graphic of the parts added and multiplied together to show how Medicare decides the PFS payment rate for services

MAC Locality ▲	MAC Description ◆	Locality Description ◆	GPCI Work ◆	GPCI PE ◆	GPCI MP ◆
0000000	NATIONAL	NATIONAL	1.000	1.000	1.000
0111205	NORTHERN CALIFORNIA	SAN FRANCISCO-OAKLAND-BERKELEY (SAN FRANCISCO CNTY)	1.082	1.374	0.452
0111206	NORTHERN CALIFORNIA	SAN FRANCISCO-OAKLAND-BERKELEY (SAN MATEO CNTY)	1.082	1.374	0.452
0111207	NORTHERN CALIFORNIA	SAN FRANCISCO-OAKLAND-BERKELEY (ALAMEDA/CONTRA COSTA CNTY)	1.082	1.374	0.452
0111209	NORTHERN CALIFORNIA	SAN JOSE-SUNNYVALE-SANTA CLARA (SANTA CLARA CNTY)	1.098	1.409	0.417
0111251	NORTHERN CALIFORNIA	NAPA	1.051	1.265	0.513
0111252	NORTHERN CALIFORNIA	SAN FRANCISCO-OAKLAND-BERKELEY (MARIN CNTY)	1.082	1.374	0.487
0111253	NORTHERN CALIFORNIA	VALLEJO	1.051	1.265	0.487
0111254	NORTHERN CALIFORNIA	BAKERSFIELD	1.027	1.079	0.694
0111255	NORTHERN CALIFORNIA	CHICO	1.021	1.079	0.579

Your Compliance Toolkit

Official CMS Resources: MLN Matters, Medicare Claims Processing Manual.

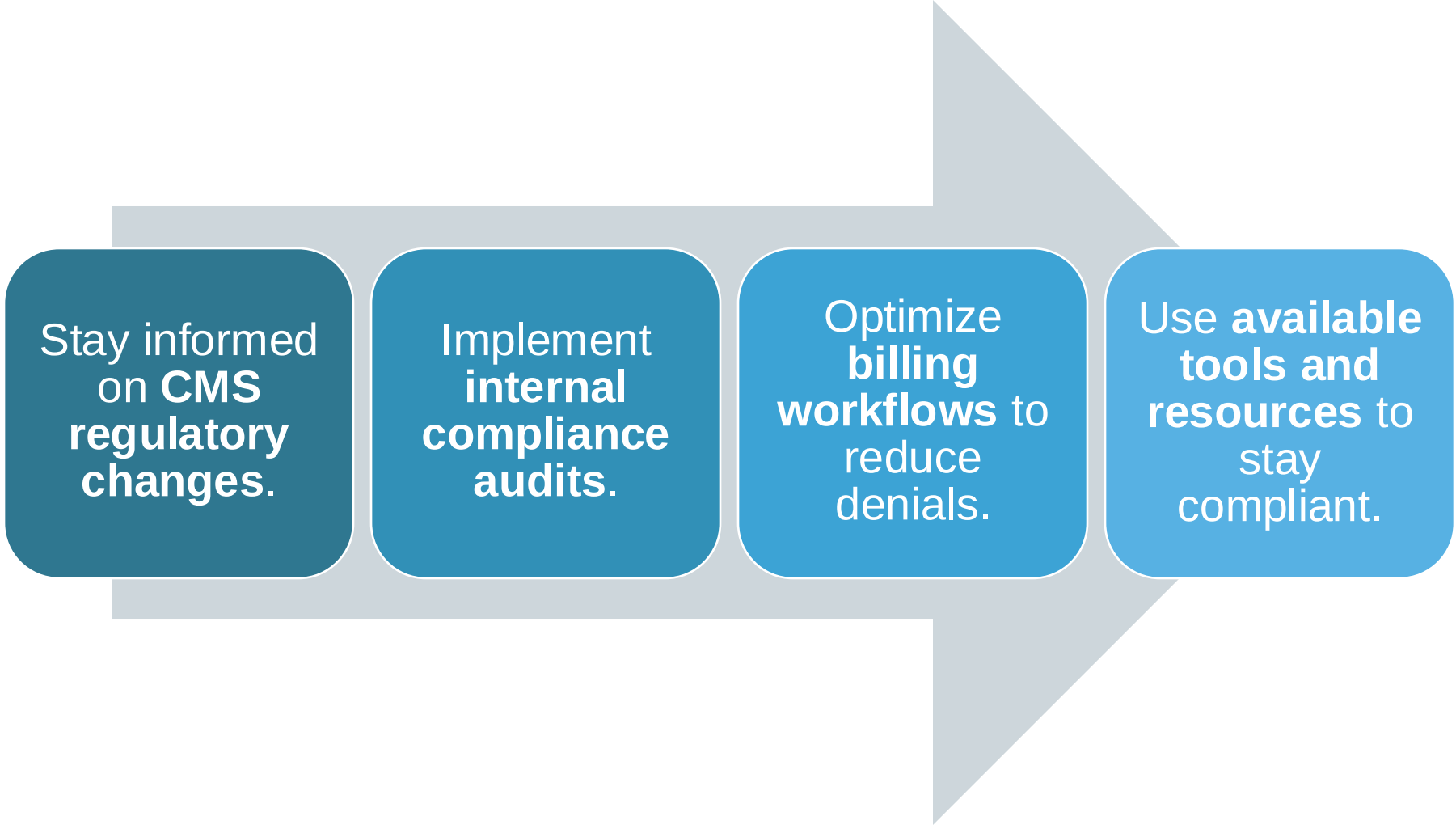
Industry Tips: AAPC, AHIMA, OIG Work Plan.

Coding Audit Software & Tools: EncoderPro, Audit Manager.

Ongoing Education: Webinars, CMS Open Door Forums, Compliance Workshops.



What to do Next?



Stay informed
on **CMS**
regulatory
changes.

Implement
internal
compliance
audits.

Optimize
billing
workflows to
reduce
denials.

Use **available**
tools and
resources to
stay
compliant.

CONTACT US



*Thank you for investing in healthcare
advancement with us*